جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : جمهوريه عباس السيد نصر
المحافظة : الجيزه **الحي / المدينة :** اوسيم **العنوان :** ~~
رقم البطاقة : 25407162100383 **رقم التليفون :** 01155923276 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 16-07-1954 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جمهوريه عباس السيد نصر
National ID: 25407162100383

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرا عن حالته

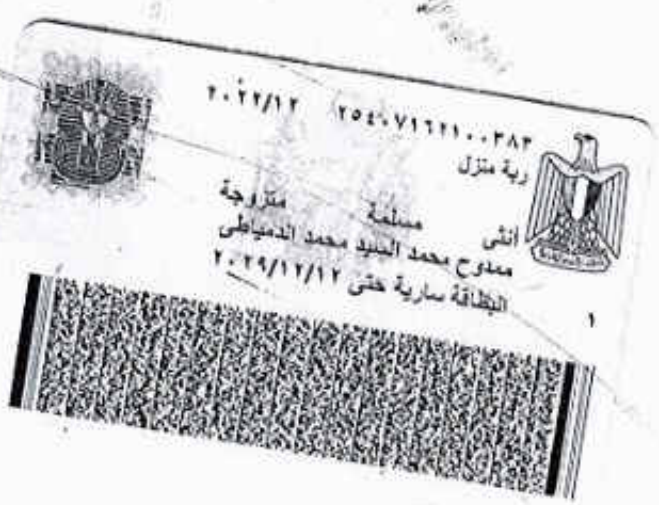
تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
جمهوريه
عباس السيد نصر
١٠ ش الحرية - بشتيل
مركز أوسيم - الجيزه
٢٥٤٠٧١٦٢١٠٠٣٨٣
JR8514648


٢٥٤٠٧١٦٢١٠٠٣٨٢
٢٠٢٢/١٢
ربة منزل
انثى
مسلمة
متزوجة
ممدوح محمد السيد محمد الدمياطى
البطاقة سارية حتى ٢٠٢٩/١٢/١٢

Patient Name: Ghadeer Abbas Al Sayed Nasr	Date: 15/02/2024
Exam: MAMMOGRAPHY BOTH	Age: 50 Year(s)
Medical ID: MR14262	Par ID: 4355

LEFT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- left breast almost entirely fatty (ACR A)
- No spiculated masses or grouped microcalcifications
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retroareolar ducts
- Homogeneous parenchyma with preserved fatty tissue and uniform cortical thickness
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supraclavicular regions

Impression

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

15/02/2024 3:04 PM

Mammogram
[DIGITAL MAMMOGRAM]

Name: جمهوية سيد عباس نصر	
MR. : 8596	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نتشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient has a diagnosis of IDC GII CT1 N2 M0 ER PR -VE HER2 3+. Neoadjuvant chemotherapy (NACT) was administered followed by surgery, and the patient is scheduled to receive adjuvant anti-HER2 therapy with osteoprotocol. A DEXA scan is pending, and the patient will receive Zometa for 6 months. Current plan includes laboratory workup, CA (likely CA-125 or CA-15-3), vitamin D supplementation, and paracetamol as needed.

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	
الطبيب المعالج :	هنا اسامه عبد الرحيم عبد البر		

يعتمد ,

Patient Name	: [redacted]	Request No.	: 13005723
Patient No.	: 25407152100383	Request Date	: 03/11/2022
Pat. No.	: 3301/22	Verification Date	: 20/11/2022
Sex	: Female	Financial Category	: اورام دار السلام - نقابة الدولة
Age	: 68 Y	Department	: Out Patient
Referring Doc.	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Right breast cancer, (post neoadjuvent therapy)

Nature of the specimen:
Right MRM

Gross Examination:
Two specimens were received submitted as:
1-Right MRM measuring 22x12x5 cm, with skin covering; 19x8 cm. Cut section revealed an ill-defined firm grsyish white tumor bed site measuring 2.5x2x0.5 cm. Dissection of axilla revealed 6 nodules ranging in size up to 1 cm in diameter.

Microscopic examination:
- No remnants of tumor tissue
- Complete pathological response
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: Free (0/6)
- Resection Margins: Free
- Histologic response to therapy: Complete pathological response.
- Additional pathologic findings: Hyalinosis, Fibrosis (Effect of therapy)

Diagnosis:
Right breast cancer, Right MRM (post neoadjuvent);
No remnants of tumor tissue in apreviously diagnosed carcinoma.
Complete pathological response
Free resection margins.
No lymphatic tumor emboli

Verified by :
Dr.Marwa mahmoud

Pathology Report
[DOCTYPE28 PATHOLOGY_REPORT]

Patient Name	: [illegible]		
Patient No.	: 25407162100383	Request No.	: 13005723
Path No.	: 3301/22	Request Date	: 03/11/2022
Sex	: Female	Verification Date	: 20/11/2022
Age	: 68 Y	Financial Category	: اورام دار السلام - تغطية الدولة
Referring Doc :		Department	: Out Patient
		Registered by	: .marwa mohamed youss

Pathology Report

No Perineural invasion
Free lymph nodes (0/6)
Stage: ypT0 ypN0

Prof. Aml Hareedy

Verified by :
Dr.Marwa mahmoud

Pathology Report
[DOCTYPE24 THCOLOR1 REPORT]

NASSER INSTITUTE
ONCOLOGY CENTER

NUCLEAR MEDICINE UNIT

Patient Name: Gowhoria Abbas Elsaid Date: 15/6/2022

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

- No definite hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Degenertaive changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are normally seen

Conclusion:

- *The surveyed skeleton is free of any active osseous lesions .*

Thanks for reference

Nuclear Medicine Unit

DR/Omnia Talaat,MD

DR/Sherine Mahdy,MD

Ministry Of Health
Specialized Medical Centers
Dar El Salem Cancer Hospital
Radiology Department

وزارة الصحة والسكان
ادارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Gmhorih Abbas Alfabed	Date : 06/08/2022
Exam : C.T CHEST	Age : 68 Year(s)
Medical ID: 2022/1981	Pat. ID: 16355

NON-CONTRAST MULTI- SLICE CT SCAN OF THE CHEST: -

A known case of breast cancer.

Findings:

- Left lower lobe apical segment pneumatocele.
- Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Upper abdominal cuts are unremarkable.

OPINION:

- ***Essentially normal CT study of the chest.***

Dr. Amr Farouk, M.D.

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 20 / 11 / 2022 G

Name:		MRN	
		ID	
Age:		Phone	
Menopausal status:		Marital status	
MRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: MRM

ER:	PR:	HER 2:	KI67:
8			81.

Sonomammography:

after 6 cycles NAC

Mets Workup: free

= Complete pathological response

= Modified Bloom

R L

TNM:

ypT0 ypN0

Recommendations:

Consider adjuvant radiotherapy

MDT OUTCOME & PLAN

4 / 12 1) PORT

2) PHESGO completion 17 Cycles

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		20/11/22			

MDT-CSM-F-001-001
Revision No: C | 10/3
Date of issue: 4/5/2022

Name : Gomohria Abbas Date : 30-05-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast UOQ irregular speculated mass lesion measuring 2 X 1.7 cm which appears fairly separable from the right pectoral muscle.
- Prominent right axillary level I and II lymph nodes the largest measures 1.1 cm.
- Few bilateral scattered pulmonary nodules ranging 3-4 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is mildly enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Bilateral renal cortical cysts.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Medical No. : 10545

Patient's Name: Jumhūrīyah Abbas El-Sayed Nasr

Date: 2026-10-11

Referred by: Prof.Dr. **no doctor**

Hospital: Goustave Roussey International Egypt

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, for routine follow up.

Findings:

Left breast.

- Almost fatty breast density (ACR A)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.
-

Conclusion:-

- ***Left breast (BIRADS 1): normal study.***
- ***Clear right mastectomy bed and axilla.***

Recommendation:-

- ***For annual follow up.***

Best Regards,

DR. Nehal Fouda,MD.

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Date : 02/11/2025	Patient Name: Nagwa Salama Ahmed Mohamed
Age : 52 Year(s)	Exam : MAMMOGRAPHY BOTH
Pat. ID: 30634	Medical ID: 4403/2020

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: right MRM & ALAND, for , routine follow up.

Findings:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- **A small rounded circumscribed hypoechoic lesion , noted at lateral aspect of the scar at mid axillary line , measures 10 x 8 mm , with mild color uptake on doppler study ?? pathological LN .**
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:-

- ***Left breast (BIRADS 1): normal study.***
- ***SUSPICIOUS right mastectomy bed lesion .***

Recommendation:-

- ***For sonar guided biopsy***

Best Regards,

DR, RANIA AHMED , MD

AM 11:13 2025/11/

Medical No. : 10545
Patient's Name: Jumhūrīyah Abbas El-Sayed Nasr
Date: 2026-10-11
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Enlarged in size (16.8cm at Rt MCL) bright parenchymal echo pattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- *surgically removed*
- *CBD is not dilated.*

Spleen: -

Average size, homogenous.

- **Kidneys: -**
- *Both are average in size, shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *right renal simple cyst 3.5cm*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Not distended enough however no stones or diverticular out pouching*

Uterus and adnexal: *no abnormalities could be detected*

Opinion:

Bright hepatomegaly likely fatty
Cholecystectomy
Right renal simple cysts
Marked colonic gaseous distention

DR: Nermeen Ramadan.MD

MDT FORM

Name	
Age	
MRN	
ID	
Co morbidity	

Clinical Data

Bx: IDC GII

SM:

Mets Workup:

IHC:
ER: -ve
PR:
Her2: +3
Ki67:

COMMENT: 68 y old F c Rt T1 N2 M0 Bilat CA
CT chest pulm nods nodal F-t Roll-over up
received 1 w. T3N1 + 3 HP c/ + ve lymph nodes T3 cN1
clinically Bilat mass n/l Rt but still operable
2/2 cm Fit possibility I Surgery

CTNM
1 2 3a

MDT OUTCOME & PLAN

15/10/2022 → considering Gr 3 toxicities / Comorbid
⊕ v. good clinical response
⇒ for → Breast wise on
• Rt BCS + ALND

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

SIGNATURE

15/10/2022

١٩٨١
cc

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name:
Date: 6/15/2023

EF	50%	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.8	2-3.7cm
LVESD	3.4	2-4cm	LA	4.1	2-4cm
FS	25%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions N ← & fair systolic fnct

LV wall thickness N ←

LV diastolic function DD III

SWMA Mild Global HK

LA and Aortic root dimensions Dilated LA

Mitral Valve Severe MR

Aortic valve Sclerotic & N ← flow

Tricuspid valve Mild TR, RVSP 25 mm Hg

Pulmonary valve N ←

Right side and systolic function N ←

Intracardiac masses and effusion No by TTE

Dr . Ahmed Abdellatif

Msc cardiology

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مركز اورام دار السلام

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

اسم المريض رباعي : محمود عبدالله
رقم الملف الطبي : 1111
نوع الوجبة :
تاريخ اعداد الوجبة : 18
وقت اعداد الوجبة :

port

EF	61	55-75%	PWT	0,8	0.6-1.1cm
LVEDD	5,2	3.5-5.8cm	AO R	2,3	2-3.7cm
LVESD	3,5	2-4cm	LA	2,6	2-4cm
FS	33	25-45%	RV	/	0.7-2.3cm(supine)
SWT	0,8	0.6-1.1cm	AO CUSP		1.5-2.6cm

LV dimensions Normal

LV wall thickness normal

LV diastolic function D.DT

SWMA Non

LA and Aortic root dimensions normal

Mitral Valve normal

Aortic valve normal

Tricuspid valve Trivil TR

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

D/ Msc cardiology

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

١٩٨١
cc

Echo report

Name: جهود ريم عبد السلام /p
Date: 20/5/ 2023

EF	50 %	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	5.2	3.5-5.8cm	AO.R	2.5	2-3.7cm
LVESD	3.9	2-4cm	LA	3.0	2-4cm
FS	25 %	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions N ——— (Fair Systolic fnct ———

LV wall thickness N ———

LV diastolic function DDI

SWMA Mild global hypokin.

LA and Aortic root dimensions N ———

Mitral Valve Sclerosis MR

Aortic valve N ———

Tricuspid valve Mod TR, RVSP 35 mmHg

Pulmonary valve N ———

Right side and systolic function N ———

Intracardiac masses and effusion No by TTE

Dr . Ahmed Abdellatif

Msc cardiology

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مركز اورام دار السلام

Echo report

Name: م/ محمود رمضان عباس السيد
Date : 28/2/2023

EF	58%	55-75%	PWT	1.1	0.6-1.1cm
LVEDD	4.3	3.5-5.8cm	AO.R	2.4	2-3.7cm
LVESD	2.7	2-4cm	LA	3.8	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine
SWT	1.3	0.6-1.1cm	AO.CUSP		1.5-2.6cm

- Concentric LVH.

~~normal~~ systolic function with ~~normal~~ internal dimension EF 58%

impaired diastolic function. grade II

~~normal~~ LA dimensions

Normal Ao root dimensions.

~~normal~~ Mitral Valve morphology . Moderate Posteriorly directed MR.

sclerotic Aortic valve morphology , No gradient.

~~normal~~ Tricuspid valve morphology and leaflets

Normal Pulmonary Valve.

No intrcardiac masses or pericardial effusion

Conculsion: . Picture of hypertensive heart d.s
. Moderate MR

DR. Mohammed A. Wahab

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام
قسم الايكو

Echo report

MR : ١٩٨١/cc

Name:
Date: ٢٤/١/ 2023

EF	58%	55-75%	PWT	1	
LVEDD	4.2	3.5-5.8cm	AO.R	2.6	2-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.2	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions N

LV wall thickness increased

LV diastolic function DD grade I

SWMA No SWMA

LA and Aortic root dimensions N

Mitral Valve Trivial MR

Aortic valve N

Tricuspid valve Trivial TR

Pulmonary valve N

Right side and systolic function N

Intracardiac masses and effusion No

Normal L.V systolic dimension & function EF = 58%. DD grade I

Dr. Msc cardiology

وزارة الصحة والسكان
مائة المستشفى الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name: السيد/ محمود
Date: 27/7/2023

MR 19811/cc

EF	66%	55-75%	PWT		0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	3.5	2-3.7cm
LVESD	3.06	2-4cm	LA	3.7	2-4cm
FS	36%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function DD GI

SWMA No RWMA at rest

LA and Aortic root dimensions

Mitral Valve Calcific with N-1 flow

Aortic valve Sclerocalcific without flow

Tricuspid valve mild TR

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion No

D/ Mohamed Ali

Msc cardiology

Patient Name	: الأستاذه / جمهوريه عباس السيد نصر	Request No.	: 13005723
Patient No.	: 25407162100383	Request Date	: 03/11/2022
Path No.	: 3301/22	Verification Date	: 20/11/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 68 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Right breast cancer, (post neoadjuvent therapy)

Nature of the specimen:
Right MRM

Gross Examination:
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1-Right MRM measuring 22x12x5 cm, with skin covering; 19x8 cm. Cut section revealed an ill-defined firm grsyish white tumor bed site measuring 2.5x2x0.5 cm, Dissection of axilla revealed 6 nodules ranging in size up to 1 cm in diameter.

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- Ductal carcinoma in situ: No
- Lymph node involvement: Free (0/6)
- Resection Margins: Free
- Histologic response to therapy: Complete pathological response.
- Additional pathologic findings: Hyalinosis, Fibrosis (Effect of therapy)

Diagnosis:
Right breast cancer, Right MRM (post neoadjuvent);
No remnants of tumor tissue in apreviously diagnosed carcinoma.
Complete pathological response
Free resection margins.
No lymphatic tumor emboli

Verified by :
Dr.Marwa mahmoud

Room No. (4) - First Floor Building No. 937- Nile Corniche - Mor El Qadeem - Cairo, Egypt

Patient Name	: الأستاذه / جمهوريه عباس السيد نصر	Request No.	: 13005723
Patient No.	: 25407162100383	Request Date	: 03/11/2022
Path No.	: 3301/22	Verification Date	: 20/11/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 68 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

No Perineural invasion
Free lymph nodes (0/6)
Stage: ypT0 ypN0

Prof . Aml Hareedy

Verified by :
Dr.Marwa mahmoud

شقة رقم (4) - الدور الأول - عمارة رقم (927) - شارع النيل - مصر القديمة - القاهرة - مصر
Room No.(4) - First Floor Building No. 927 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / جمهوريه عباس السيد نصر	Request No.	: 13002533
Patient No.	: 25407162100383	Request Date	: 11/05/2022
Path No.	: 1215/2022	Verification Date	: 28/05/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 67 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Rt breast UOQ mass

Nature of the specimen: Tru-cut biopsy, collected at 10/5/2022, 01:59 pm.

Gross Examination:
- Procedure: Tru-cut biopsy: (Four greyish white and fatty tissue cores, 1 cm each + multiple hemorrhagic fragments, totally submitted.)
- Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
• Histological type: Invasive Ductal Carcinoma
• Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1)
• Biomarkers:
ER: No nuclear reaction in tumor cells (Allred score 0/8)
PR: No nuclear reaction in tumor cells (Allred score 0/8)
Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)
Ki 67: 8 % nuclear reaction

Diagnosis:
Right breast UOQ mass, Tru-cut biopsy:
- Invasive Ductal Carcinoma GII
- ER : Negative (0/8)
- PR : Negative (0/8)
- HER2 : (Positive score 3+)
- KI67 : 8% nuclear reaction

قسم الباثولجي

Verified by :
walaa ghoniem 28/5/2022

Patient name: Gomhorya Abbas

Date:10/5/2022

Combined bilateral breast mammography and complementary ultrasound

Indication:

Left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR. A)
- Right breast UOQ zone c irregular spiculated high density mass.
- Complementary ultrasound showed right breast UOQ irregular spiculated hypoechoic mass, with internal vascularity on color Doppler application, seen at 11 o'clock, 0.6 cm from the overlying skin, measured 2 x 1.6 cm, consistent to the palpable lump.
- No left breast suspicious cystic or solid mass lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific axillary LN.
- Pathological right axillary, level I LNs with focal cortical thickening largest measures 0.6 x 1.6 cm.

Opinion:

- Right breast 11 o'clock highly suspicious mass with level I pathological lymphadenopathy (BIRADS 4), for histopathological correlation.
- Normal left breast study (BIRADS 1)

Yours truly

Dr Aya Deabes,MSc.

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	الأستاذه / جمهوريه عباس السيد نصر	Request No.	13002533
Patient No.	25407162100383	Request Date	11/05/2022
Path No.	1215/2022	Verification Date	28/05/2022
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	67 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Prof. Lubna Omar
Dr. Menna Atef

Dr. Walaa Ghonaim

28/5/2022

قسم الباثولوجي

Verified by :
walaa ghoniem

Room No. (4) - First Floor-Building No. (87)- Nile Corniche - Mit El Qadema- Cairo, Egypt

اسم الفـــرع بشتيل
مدير الوحده ا.د. عزة يونس
طبيب الوحده د. محمد عبد الحافظ

مدير المعامل
ا.د. فاطمة نصرت

Visit Number:	52622506849	Registered	14-08-2022 11:59:24
Patient Name:	جمهورية عباس السيد	Collected	14-08-2022 12:00:02
Age / Sex:	64 Year / Female	Authenticated	14-08-2022 19:15:55
Referred By:	Prof : -	Printed	18-08-2022 09:19:11
Client Name:	0		

TEST NAME	RESULT	UNIT	REFERENCE RANGE
Stool Analysis			
Colour	Brown		Brown
Odour	Offensive		Feacal
Consistency	Loose		Semi - formed
Mucus	Few		Nil
Blood	Nil		Nil
pH	Alkaline		Alkaline
Worms	Nil		Nil
Undigested Food	Few		Nil
Vegetable Cells	Some		Nil - Few
Starch	Some		Nil - Few
Muscle Fibers	Nil		Nil - Few
Fat	Few		Nil - Few
Pus Cells	15-20	/H.P.F	0 - 1
RBCs	6-8	/H.P.F.	0 - 1
Epithelial Cells	Nil		Nil
Yeast Cells	Many		Nil
Protozoa	Few Entamoeba coli cysts		Nil
Helminths	Nil		Nil

Comment : Stool culture is recommended.

Dr.Azza Younis
Professor of Parasitology,
Faculty of Medicine, Cairo University

A.Younis

Scan Me

اسم الفرع بشتيل
مدير الوحده د. غادة عبدالتواب
طبيب الوحده د. محمود حسين
د. رنا محمود
د. فاطمة ربيع

مدير المعامل
ا.د. فاطمة نصرت

Visit Number:	52622506849	Registered	14-08-2022 11:59:24
Patient Name:	جمهورية عباس السيد	Collected	14-08-2022 12:00:02
Age / Sex:	64 Year / Female	Authenticated	16-08-2022 14:21:54
Referred By:	Prof : -	Reported	18-08-2022 21:19:13
Client Name:	0		

Stool Culture and Sensitivity

Aspect [stool]	**Loose**
PUS cel /H.P.F.	**15-20**
Protozoa	**Few Entamoeba coli cysts**
Culture	**Negative for Salmonella and Shigella spp.**

Scan Me

Dr.Ghada Abd El Tawab
M.D. Clinical & Chemical Pathology,
Faculty of Medicine, Al Azhar University

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital
معهد ناصر
مركز الأورام

Oncology Center
مركز الأورام

NASSER INSTITUTE
ONCOLOGY CENTER
خطابات أورام

NUCLEAR MEDICINE UNIT
وحدة الطب النووي

Patient Name: Gomhoria Abbas Elsaid *Date: 15/6/2022*

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No definite hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Degenertaive changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are normally seen

Conclusion:

- ***The surveyed skeleton is free of any active osseous lesions .***

Thanks for reference

Nuclear Medicine Unit

DR/Omnia Talaat,MD ***DR/Sherine Mahdy,MD***

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٣٣٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel. 22039164 - 22039356 Fax . 22039 236

Patient Name: Gmhorih Abbas Al Sayed Nasr	Date : 01/10/2022
Exam : ultrasound ABDOMEN& PELVIS	Age : 68 Year(s)
Medical ID:	Pat. ID: 12142

Patient's Name : Gomhoria Abbas Elsayed
Date : 1 October 2022

ABDOMINO-PELVIC SCAN OF YOUR PATIENT REVEALED:

LIVER

- Mildly enlarged size measuring 16.7 cm showing bright coarse parenchymal echo-pattern, smooth outlines & normal hepatic veins. Average portal vein caliber with patent lumen. No focal lesions.
- ***Average calibre CBD is 0.6 cm with prominent central intra-hepatic biliary radicles.***
- No ascites.

GALL BLADDER

- Surgically removed.

SPLEEN: average size with normal echogenicity & no focal lesions.

KIDNEYS

- Normal size, parenchymal thickness and echogenicity.
- No stones or backpressure changes could be noted.
- ***Bilateral renal cortical simple cysts; the largest on the right side measuring 3 x 2.6 and the largest on the left side 1.8 x 1.4 cm.***

URINARY BLADDER: empty at time of examination.

Colonic gas distension.

OPINION:

⇒ ***Bright coarse liver with prominent central intrahepatic biliary radicles for lab correlation.***

Dr/Lamiaa Elsharkawy, MD.

Signature Dr. Lamyaa Nagah

01/10/2022 1:49 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Gmhorih Abbas Al Sayed Nasr	Date : 15/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 69 Year(s)
Medical ID: 1981/2022	Pat. ID: 16355

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- left breast almost entirely fatty (ACR A)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

15/02/2024 3:04 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Gmhorih Abbas Aliabed	Date : 06/08/2022
Exam : C.T CHEST	Age : 68 Year(s)
Medical ID: 2022/1981	Pat. ID: 16355

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

A known case of breast cancer.

Findings:

- Left lower lobe apical segment pneumatocele.
- Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Upper abdominal cuts are unremarkable.

OPINION:

- ***Essentially normal CT study of the chest.***

Dr. Amr Farouk, M.D.

Name : Gomohria Abbas **Date :** 30-05-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. **Chest:-**

- Right breast UOQ irregular speculated mass lesion measuring 2 X 1.7 cm which appears fairly separable from the right pectoral muscle.
- Prominent right axillary level I and II lymph nodes the largest measures 1.1 cm.
- Few bilateral scattered pulmonary nodules ranging 3-4 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. **Abdomen and pelvis:**

- The liver is mildly enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Bilateral renal cortical cysts.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Dar El-Salam Caner Center
Radiology Department

مركز أورام دار السلام
قسم الأشعة

Patient's Name : Gomhoria Abbas
Date : 6 August 2022

COLOURED DUPLEX DOPPLER STUDY OF IJV & SUBCLAVIAN VEINS :

- **Right side:**
  - IJV: diameter 11 mm showing patent lumen & normal monophasic flow.
  - Subclavian vein: patent with diameter 6.7 mm & normal monophasic flow.

- **Left side:**
  - IJV: diameter 9 mm showing patent lumen & normal monophasic flow.
  - Subclavian vein: patent with diameter 6 mm & normal monophasic flow.

Dr. Lamiaa Elsharkawy, MD